



EVERGREEN COMMUNITY CARE INC.

RESTRAINT AND SECLUSION POLICY

Emergency Use of Physical Restraint · Prohibition of Seclusion · MCFD Standards for Staffed Children's Residential Services (SHSS), Standard D.5, October 2016 Amendment

Document Reference	ECC-RS-2026-001
Version	1.0 — July 2026
Approved By	Executive Director
Effective Date	July 1, 2026
Review Date	July 1, 2027 (annual); also immediately upon any funder directive, reportable incident involving restraint, or BC legislative/regulatory change
Applies To	All Evergreen Community Care residential care programs and 1–2 bed homes serving children and youth in MCFD care. This policy governs the MCFD stream only. Restrictive interventions for adults receiving CLBC-funded services are governed by the Behaviour Support Policy & Procedure Manual (ECC-BSP-2026-001) and the CLBC Behaviour Support and Safety Planning Policy (2024).
Funder Streams	M = MCFD (children and youth). This policy does not apply to the CLBC (adult) stream — see ECC-BSP-2026-001 for adult restrictive-practice provisions.
Legislative Authority	MCFD Standards for Staffed Children's Residential Services (SHSS), Standard D.5, as amended October 2016; Child, Family and Community Service Act (CFCSA); Residential Care Regulation (RCR) ss.73–80; Workers Compensation Act (WCA) and Occupational Health and Safety Regulation; BC Human Rights Code; Representative for Children and Youth Act
Funder Standards	MCFD SHSS Standard D.5 (behaviour support, restraint and prohibition of seclusion, as amended October 2016); MCFD reportable-circumstances requirements; RCR ss.73–80 (use of restraint in a community care facility)
Related Documents	Behaviour Support Policy & Procedure Manual (ECC-BSP-2026-001) — this policy operates as a companion to, and does not replace, the behaviour support and safety planning framework in ECC-BSP; Illness, Injury & Medical Emergency Manual (ECC-IIME-2026-001) — reportable circumstances; HR Policy Binder (ECC-HR-POL-2026-001) — training records; Individual Service Plan Process Manual (ECC-PC-MAN-2026-001)

⚠ This policy governs the use of physical restraint as an emergency, last-resort intervention only. Seclusion — the involuntary confinement of a child or youth alone in a room or area from which they are physically prevented from leaving — is PROHIBITED in all Evergreen Community Care programs, consistent with the October 2016 amendment to MCFD SHSS Standard D.5. Restraint is never used as punishment, for staff convenience, or as a substitute for a behaviour support plan. Every use of restraint is a reportable circumstance. Any question about whether an intervention is permitted is escalated to the Program Coordinator or Program Manager before it is used, wherever the situation allows time to do so.

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PART 1 — Policy Foundation

1. Policy Statement and Governing Philosophy

Every child and youth in an Evergreen Community Care (ECC) program has the right to be safe, to be treated with dignity, and to be free from unnecessary physical intervention. ECC's approach to behaviour is built on relationship, prevention, and the individualized Behaviour Support Plan developed for each child or youth under ECC-BSP-2026-001. Restraint is not a behaviour-management tool; it is an emergency safety response used only when a child or youth's behaviour presents an imminent risk of serious physical harm to themselves or another person, and only when less restrictive strategies have failed or cannot be safely attempted.

This policy implements MCFD Standard D.5 as amended in October 2016, which narrowed the circumstances in which restraint may be used in staffed residential resources and prohibited seclusion outright. ECC's practice meets or exceeds this standard in every program, in every home, for every child and youth, without exception.

This policy is operational; it governs what staff do in the moment and what is documented afterward. It does not replace clinical or legal advice, and any situation not clearly addressed here is escalated to the Program Coordinator (PC) or Program Manager (PM) before further action is taken, where time and safety permit.

2. Legislative and Regulatory Framework

This policy is governed by, and must be read alongside, the following:

- MCFD Standards for Staffed Children's Residential Services (SHSS), Standard D.5, as amended October 2016 — the amendment that removed seclusion as a permitted intervention and tightened the conditions under which physical restraint may be used;
- Child, Family and Community Service Act (CFCSA) — duty to protect and the best-interests-of-the-child standard;
- Residential Care Regulation (RCR), ss.73–80 — use of restraint in a licensed or designated community care setting;
- Workers Compensation Act (WCA) and the Occupational Health and Safety Regulation — safe work practices, physical intervention training, and incident reporting duties owed to staff as well as to the people in care;
- BC Human Rights Code — freedom from discriminatory or degrading treatment;
- Representative for Children and Youth Act — oversight and reporting obligations that may be engaged by a restraint incident.

Where this policy and any of the above sources appear to conflict, staff escalate to the PM immediately; the PM confirms the correct standard with MCFD before any recurring practice is adopted. This policy is reviewed at minimum annually and immediately upon any change to the SHSS or applicable legislation.

3. Roles and Responsibilities

Role	Responsibility
Youth Care Worker (YCW) / Direct Support Staff	Uses prevention and de-escalation first; uses physical restraint only when the imminent-risk test in Section 9 is met and only using an approved technique; completes the Restraint Incident Report (Appendix A) before the end of shift; participates in the post-incident debrief.
Shift Lead	Provides on-shift guidance during and immediately after an incident; confirms a medical check has occurred; ensures the Restraint Incident Report is started before shift end.

Role	Responsibility
Program Coordinator (PC)	Receives every Restraint Incident Report within 24 hours; confirms notifications (Section 16) have been made; reviews the incident against the child or youth's Behaviour Support Plan; schedules the debrief (Section 19).
Program Manager (PM)	Reviews all restraint incidents monthly; identifies patterns of repeated use (Section 20); reports to MCFD as a reportable circumstance (Section 17); ensures training currency across the program.
Executive Director (ED)	Holds overall accountability for policy compliance; reviews quarterly restraint data; approves any amendment to this policy; is notified immediately of any restraint incident resulting in injury.

4. Key Definitions

Term	Definition
Physical (manual) restraint	The use of bodily force by staff to restrict a child or youth's movement, applied only to prevent imminent serious physical harm, using an approved technique, for the shortest time necessary.
Mechanical restraint	Any device or equipment used to restrict movement. PROHIBITED in all ECC programs under all circumstances.
Chemical restraint	The use of medication for the primary purpose of restricting behaviour or movement, rather than treating a diagnosed condition. PROHIBITED. See Medication Administration Manual (ECC-MED-2026-001) for the distinction between PRN medication prescribed to treat a diagnosed condition and any use of medication to control behaviour, which is never permitted.
Seclusion	The involuntary confinement of a child or youth alone in a room or area, from which they are physically prevented from leaving, whether by a locked door, a held door, or any other means. PROHIBITED — see Part 3.
Time-out / cool-down space	A voluntary, non-locked space a child or youth may choose to use, or may be invited to use, to self-regulate, which they remain free to leave at any time. This is not restraint or seclusion and is addressed in the Behaviour Support Plan (ECC-BSP-2026-001), not in this policy.
Imminent risk	A risk of serious physical harm that is about to happen or is happening, as distinct from a risk that is possible, anticipated, or based on past behaviour alone.
Reportable circumstance	An event that MCFD requires to be reported under its reportable-circumstances framework, cross-referenced at ECC-IIME-2026-001. Every use of restraint is a reportable circumstance.

PART 2 — Prevention and the Least-Restrictive Standard

5. Prevention-First Philosophy

Restraint is a failure state to be prevented, not a routine tool. Every ECC program operates on the presumption that behaviour is communication, and that the right response to escalating behaviour is relationship, environment, and strategy — not force. Staff are trained to recognize a child or youth's early warning signs and to intervene long before a situation could become physically dangerous.

Before any physical intervention is considered, staff must have attempted, or must be able to explain why it was not safe to attempt, the de-escalation and prevention strategies specific to that child or youth as set out in their Behaviour Support Plan (ECC-BSP-2026-001), including environmental modification, redirection, offering choice, reducing demands, and creating space.

6. Relationship to the Behaviour Support Plan

This policy governs the narrow emergency circumstance in which physical restraint may be used. It does not stand alone: every child or youth's Behaviour Support Plan (BSP), developed under ECC-BSP-2026-001, must identify their individual triggers, early warning signs, and preferred de-escalation strategies, and — where restraint has ever been used or is considered foreseeably necessary — must name the specific approved technique(s) authorized for that individual and any medical or trauma-related contraindications (for example, a history that makes any hold to the torso or floor unsafe).

Staff must know the BSP of every child or youth in their care before working an unsupervised shift. A restraint used in a manner inconsistent with a documented contraindication in the BSP is treated as a serious practice concern, reviewed by the PM regardless of whether harm resulted.

7. Practices That Are Always Prohibited

The following are prohibited in every ECC program, without exception, regardless of the severity of the behaviour presented:

- Seclusion, in any form (see Part 3);
- Mechanical restraint of any kind;
- Chemical restraint — the use of medication to control behaviour rather than to treat a diagnosed condition;
- Any restraint technique that restricts breathing, applies pressure to the neck, chest, or back while prone, or that is not part of ECC's approved training curriculum;
- Restraint used as punishment, discipline, or in response to a child or youth's verbal defiance, property damage that presents no risk to a person, or refusal to comply with a direction;
- Restraint used for staff convenience or because it is faster than de-escalation;
- Withholding food, water, sleep, washroom access, or contact with a guardian or social worker as a consequence of a behaviour incident;
- Any threat to use restraint, or use of the fact that restraint is available, as a means of intimidation or compliance.

PART 3 — Prohibition of Seclusion

8. Seclusion Is Prohibited

Seclusion — confining a child or youth alone in a room or area against their will, whether by a locked door, a physically held door, furniture placed to block an exit, or any other means that prevents them from leaving — is prohibited in every ECC program, in every circumstance, with no exception for severity of behaviour, time of day, or staffing level.

This prohibition implements the October 2016 amendment to MCFD SHSS Standard D.5, which removed seclusion as a permitted behaviour-management intervention in staffed residential resources. Any practice that functions as seclusion — including holding a door closed, removing a means of exit, or verbally instructing a child or youth that they are not permitted to leave a room while physically preventing them from doing so — is a policy violation and a reportable circumstance, regardless of the term used to describe it.

This is distinct from a voluntary time-out or cool-down space (Section 4), which a child or youth chooses to use or accepts an invitation to use, and which they remain free to leave at any time. If staff are uncertain whether a specific situation would constitute seclusion, the answer is to treat it as prohibited and to seek PM guidance immediately afterward.

A bedroom or other room may still be used as a voluntary calming space, and a child or youth may be encouraged toward it, but the door is never locked or held shut, and no furniture, staff body, or object is used to block the exit.

PART 4 — Emergency Use of Physical Restraint

9. When Restraint May Be Used — the Imminent Risk Test

Physical restraint may only be used when ALL of the following conditions are met:

- The child or youth's behaviour presents an imminent risk of serious physical harm to themselves or to another person — not a possible, anticipated, or past risk;
- Verbal de-escalation and the strategies identified in the child or youth's Behaviour Support Plan have been attempted and have not resolved the risk, or the situation is moving too fast to attempt them safely;
- No less restrictive option is reasonably available in the moment;
- The restraint is applied using an ECC-approved technique by a staff member currently certified in that technique (Part 8);
- The restraint is applied only for as long as the imminent risk continues, and is released the moment it no longer does.

If any one of these conditions is not met, restraint is not used. Property damage alone, without risk of serious physical harm to a person, does not meet this test.

10. Restraint Methods — Permitted and Prohibited

Only physical (manual) restraint techniques included in ECC's approved training curriculum (Part 8) may be used. Mechanical and chemical restraint are prohibited under all circumstances (Section 7).

Prohibited techniques include any hold that restricts breathing or applies pressure to the neck, chest, throat, or back while a child or youth is prone (face-down); any hold not included in current staff training; and any hold specifically contraindicated in the child or youth's Behaviour Support Plan.

11. Duration, Monitoring and Safety Checks

A restraint is held for the shortest time necessary to remove the imminent risk, and is continuously reassessed while it is applied. While a restraint is in progress:

- A second staff member, wherever staffing allows, monitors the child or youth's breathing, colour, and level of distress throughout, separate from the staff applying the hold;
- Staff speak calmly to the child or youth throughout, explaining what is happening and what will end the restraint;
- The restraint is released the instant the imminent risk has passed, even if this is only a few seconds after it began;
- Any restraint approaching or exceeding the duration typically covered in training as safe (per the current approved curriculum) is treated as an escalating emergency — a second staff member calls for additional support or emergency services without waiting for the restraint to end.

12. Ending a Restraint

Staff release a restraint as soon as the child or youth is no longer an imminent risk to themselves or others, and transition immediately into calming, reassurance, and support — not debrief or discipline. The immediate priority after release is the child or youth's physical and emotional safety (Part 5), not documentation, which follows once the situation is stable.

PART 5 — Immediate Post-Incident Actions

13. Immediately After a Restraint

- Check the child or youth for visible injury and ask how they are feeling, in language appropriate to their age and understanding;
- Offer water, space, and a calm adult presence; do not require the child or youth to discuss the incident before they are ready;
- Separate the child or youth from the restraining staff member if the child or youth requests this and it is safe to do so;
- Notify the Shift Lead or on-call PC immediately, and no later than within the hour.

14. Medical Check and Wellness Follow-Up

Every restraint requires a physical wellness check by staff before the end of the shift on which it occurred, documented on the Restraint Incident Report (Appendix A). Any visible injury, complaint of pain, or loss of consciousness during the restraint requires immediate medical assessment — call 911 for any serious injury or loss of consciousness — per the Illness, Injury & Medical Emergency Manual (ECC-IIME-2026-001). A wellness check-in with the child or youth is also completed the following day by the PC or a staff member with whom the child or youth has a strong relationship.

PART 6 — Documentation, Notification and Reporting

15. Restraint Incident Report

Every use of physical restraint, regardless of duration or outcome, is documented on the Restraint Incident Report (Appendix A) before the end of the shift on which it occurred. The report is factual and observational — it describes what was seen and done, not interpretations of the child or youth's intent — and is completed by the staff member who applied the restraint, with input from any staff who monitored or witnessed it.

16. Notification Requirements

Who	Timing
Program Coordinator / on-call	Immediately, and no later than within the hour of the incident
Program Manager	Within 24 hours, via the completed Restraint Incident Report
Guardian / parent (M)	As soon as possible, and consistent with the child or youth's safety plan and any confidentiality direction on file
Social Worker	Per MCFD reportable-circumstances timelines — see ECC-IIME-2026-001 and confirm current timeline requirements with the PM
Executive Director	Immediately for any restraint resulting in injury; otherwise included in the PM's routine reporting

17. Reportable Circumstances Cross-Reference

Every restraint is a reportable circumstance under MCFD's framework. The PM is responsible for submitting the reportable-circumstance notification to MCFD within the funder's required timeline, using the process and forms set out in the Illness, Injury & Medical Emergency Manual (ECC-IIME-2026-001). The Restraint Incident Report (Appendix A) and the reportable-circumstance submission are cross-referenced by date and child/youth identifier and both are retained per the Records Management & Retention Policy (ECC-REC-POL-2026-001).

PART 7 — Rights, Debrief and Review

18. Child and Youth Rights During and After Restraint

- To be treated with dignity and to have the restraint explained to them in language they understand, both during and after;
- To be free from restraint used as punishment;
- To have their guardian and social worker informed per Section 16;
- To speak with the Representative for Children and Youth or another trusted adult about the incident if they wish;
- To have the incident reviewed if it happens more than once (Section 20);
- To raise a complaint about how a restraint was handled through the Client Complaint Resolution Procedure without fear of consequence.

19. Post-Incident Debrief

The PC leads a debrief with all involved staff within 3 business days of any restraint, using the Post-Restraint Debrief Record (Appendix C). The debrief examines what led to the incident, whether prevention strategies in the Behaviour Support Plan were followed, whether the restraint met the imminent-risk test, and what could reduce the likelihood of recurrence. Where appropriate and supportive, the child or youth is offered an opportunity to share their perspective on the incident, separate from the staff debrief.

The debrief is a learning process, not a disciplinary one, unless the review identifies a clear policy violation, in which case the PM follows the HR Policy Binder (ECC-HR-POL-2026-001) performance process.

20. Review of Repeated or Patterned Use

The PM reviews restraint data monthly across the program. Two or more restraints involving the same child or youth within a 30-day period triggers a mandatory review of that individual's Behaviour Support Plan with the clinical or behaviour support resource named in ECC-BSP-2026-001, to determine whether the plan needs to change. Any pattern involving the same staff member across multiple children or youth triggers a review of that staff member's training and practice with the PM.

PART 8 — Training and Competency

21. Mandatory Training Requirements

No staff member may work an unsupervised shift in any ECC program until they have completed ECC's approved physical intervention / crisis prevention training, covering prevention and de-escalation, the imminent-risk test in Section 9, approved restraint techniques and their limits, and this policy in full, including the prohibition of seclusion. Training is refreshed at minimum annually, and immediately following any change to this policy or to the approved technique curriculum.

22. Competency Verification and Refresher Training

The PC maintains a Restraint Training and Competency Record (Appendix D) for every staff member, tracking initial certification, refresher dates, and any additional coaching following an incident review. A staff member whose certification has lapsed is not authorized to use physical restraint and must rely on de-escalation and calling for a currently certified colleague or emergency services until recertified.

PART 9 — Quality Assurance and Oversight

23. QA Indicators and Monitoring

- Number of restraint incidents per program, per month, tracked for trend;
- Percentage of Restraint Incident Reports completed within the required shift-end timeline;
- Percentage of restraints followed by a completed debrief within 3 business days;
- Any incident involving a technique or duration outside approved training parameters;
- Any repeated-use review triggered under Section 20 and its outcome.

These indicators feed the SHSS Quality Assurance Manual (QA Tool B/C) and are reported to the ED quarterly.

24. Annual Policy Review

This policy is reviewed at minimum annually by the PM and ED, and immediately upon any amendment to MCFD SHSS Standard D.5, the RCR, or applicable legislation. The annual review considers restraint data from Section 23, any debrief findings, and any MCFD monitoring feedback, and is documented in the Version History below.

Appendices

APPENDIX A — Restraint Incident Report

Child/Youth Name / Identifier	
Program / Home	
Date and Time of Incident	
Staff Involved (applying restraint / monitoring)	
What led up to the incident (factual, observed behaviour)	
De-escalation / BSP strategies attempted before restraint	
Imminent risk presented (specific and factual)	
Restraint technique used and approximate duration	
Second staff monitoring throughout? (Y/N — explain if N)	
How the restraint ended	
Injury to child/youth or staff? (describe or state none)	
Medical check completed by / time	
Consistent with Behaviour Support Plan? (Y/N — explain if N)	
Guardian notified — by whom / when	
Social Worker notified — by whom / when	
Staff signature / date	
PC review — signature / date	

APPENDIX B — Notification and Reportable Circumstance Log

Date	Child/Youth	Notified (who)	Method / Time	Reportable Circumstance Filed (Y/N, date)

APPENDIX C — Post-Restraint Debrief Record

Date of Debrief / Incident Date	
Staff Present	
Summary of what happened	
Was the imminent-risk test met? (Y/N — explain)	
Were BSP strategies followed?	
What worked / what would help next time	
Child/youth perspective offered and captured? (Y/N)	
Follow-up actions and owner	
PC signature / date	

APPENDIX D — Restraint Training and Competency Record

Staff Name	Initial Cert. Date	Refresher Due	Last Refresher	Notes

Staff Acknowledgement

All staff must read and sign this acknowledgement before unsupervised duties and upon each annual update of this policy.

I acknowledge that I have received, read, and understood the Evergreen Community Care Restraint and Seclusion Policy (ECC-RS-2026-001), Version 1.0, effective July 1, 2026.

I understand that seclusion is prohibited in every ECC program, in every circumstance, with no exceptions.	
I understand that restraint is an emergency, last-resort intervention used only when a child or youth presents an imminent risk of serious physical harm to themselves or another person.	
I understand that mechanical restraint and chemical restraint are prohibited under all circumstances.	
I understand that restraint is never used as punishment, discipline, or for staff convenience.	
I know the Behaviour Support Plan of every child or youth in my program, including any restraint contraindications.	
I will use only ECC-approved restraint techniques, and only while currently certified in physical intervention / crisis prevention training.	
I will complete the Restraint Incident Report before the end of any shift on which restraint was used, and will notify the PC/on-call within the hour.	
I understand that every restraint is a reportable circumstance and will be reviewed, and that a pattern of repeated use triggers a mandatory plan review.	

Staff Name (print)	
Position	
Program	
Signature	
Date	
PC Signature (training current, on file) / Date	

Version History

Version	Date	Summary
1.0	July 1, 2026	Initial Restraint and Seclusion Policy: implements MCFD SHSS Standard D.5 (October 2016 amendment); prohibits seclusion outright; establishes the imminent-risk test and approved-technique requirement for emergency physical restraint; prohibits mechanical and chemical restraint; sets documentation, notification, and reportable-circumstance

Version	Date	Summary
		requirements; establishes debrief and repeated-use review process; sets training and QA requirements; 4 appendices. Cross-referenced to ECC-BSP-2026-001 (Behaviour Support Plan) and ECC-IIME-2026-001 (reportable circumstances). Closes the Restraint and Seclusion Policy gap identified at item 3.6 of the MCFD/CLBC Requirements Register (ECC-REQREG-2026-001).

Executive Director Signature: _____ Date: _____
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